

SPARTAN COACHING

HOSPICE SALES EXCELLENCE

Cold Call Opening Script & Discovery Framework

PART 1: THE OPENING (First 30 Seconds)

The 30-Second Formula

"Hi [Name], this is [Your Name] with Spartan Coaching. I know you're busy, so I'll be brief. We work with facilities like yours to improve patient outcomes and family satisfaction by connecting eligible patients with hospice care earlier in their journey. I'm not selling anything today—just want to understand your situation. Do you have 30 seconds?"

Why This Works:

- Respects their time (validates their concern)
- Names the outcome upfront (better care, satisfaction)
- Removes sales pressure ("not selling")
- Creates urgency ("earlier in their journey")
- Asks for micro-commitment (30 seconds)
- Positions you as helpful, not pushy

If They Say "Yes" (Your Bridge)

"Perfect. Here's what I've observed: Many facilities like yours have eligible patients in house, but the referral process isn't structured, so some miss the window. We work with facilities to clarify that process and get better outcomes. Does that resonate with you at all?"

If They Say "No" (Your Pivot)

"I understand. Can I ask—out of curiosity—when a patient in your facility becomes appropriate for hospice, what does that referral process look like? Just trying to understand if it's something we could eventually help with."

PART 2: DISCOVERY QUESTIONS (Your Conversation Framework)

Tier 1: Initial Assessment

1. "How many patients would you estimate are in your facility at any time?"
 - > Establish baseline census
2. "Of those patients, how many would you say are appropriate for hospice?"
 - > Gauge current awareness and referral volume
3. "Do you have a formal hospice referral process, or does it vary by department?"
 - > Identify process clarity (or lack thereof)
4. "Who typically initiates the hospice conversation in your facility—nurses, doctors, care managers?"
 - > Understand decision-maker roles

Tier 2: Deeper Opportunity

5. "In the last quarter, how many patients transitioned to hospice from your facility?"
 - > Get specific referral numbers (for gap analysis)
6. "Did any patients who could have benefited from hospice not get referred? What happened?"
 - > Uncover barriers and missed opportunities
7. "How involved is your medical director in those decisions? Supportive?"
 - > Gauge physician alignment (critical for referrals)

PART 3: HANDLING OBJECTIONS (Spartan Approach)

"We already work with [Competitor]."

"I respect that. Many best-in-class facilities actually work with 2-3 partners to ensure they're not leaving eligible patients on the table. Our approach is different because... [specific differentiator]. Would it make sense to have a brief conversation

about how we complement what you're already doing?"

[Acknowledge, position as complementary, ask permission]

"We don't have a need right now."

"I hear that. Here's what I've seen: most facilities realize they DO have a gap when they look at their data. Could I just ask—if you looked at your last 20 discharges, what percentage went to hospice versus home health or other settings? That might tell us if there's an opportunity."

[Reframe from subjective to objective (data-driven)]